SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

AT 50/75

Locally advanced or metastatic breast cancer · NCCP 00423 · version 5 · 21-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
Treatment to control cancer and symptoms, with regular review of benefit and side effects.

Schedule
Doxorubicin followed by docetaxel on Day 1 every 21 days, for up to six cycles unless progression or unacceptable toxicity changes the plan.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Required blood tests and cardiac monitoring discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Doxorubicin
ANTHRACYCLINE CHEMOTHERAPY

- ☐ Low counts, infection and mouth ulcers
- ☐ Nausea, hair loss and fatigue
- ☐ Heart damage, sometimes delayed
- ☐ Severe tissue injury if the infusion leaks
- ☐ Rare later blood cancers

Docetaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts, infection and sepsis
- ☐ Hair loss, mouth soreness and fatigue
- ☐ Numbness / tingling; sometimes lasting
- ☐ Fluid retention and nail changes
- ☐ Allergic infusion reaction

Use this information to discuss the treatment goal, alternatives, phase changes, important risks and monitoring with your team. It supports the approved local consent process.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00423 v5.

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